

Pharmacode position may change as per Supplier's m/c requirement &additional
small pharma code may appear on the front / back panel



SUMMARY OF PRODUCT CHARACTERISTICS

OLANGEM OD
Olanzapine Orodispersible Tablets
5 mg and 10 mg

Rx Only

NAME OF DRUG PRODUCT: Olanzapine Orodispersible Tablets 5 mg
Olanzapine Orodispersible Tablets 10 mg

(TRADE) NAME OF PRODUCT: OLANGEM OD 5
OLANGEM OD 10
5 mg & 10 mg.

STRENGTH: Olanzapine Orodispersible Tablets 5 mg
PHARMACEUTICAL DOSAGE FORM: Orodispersible Tablet.

ROUTE OF ADMINISTRATION: Oral use.

QUALITATIVE AND QUANTITATIVE COMPOSITIONS:
Olanzapine Orodispersible Tablets 5 mg
Each Orodispersible tablet contains Olanzapine Ph.Eur 5 mg.
Olanzapine Orodispersible Tablets 10 mg
Each Orodispersible tablet contains Olanzapine Ph.Eur 10 mg

PHARMACEUTICAL FORM:
Olanzapine Orodispersible Tablets 5 mg: Yellow colored, circular, flat faced beveled edge tablets, debossed with "C" on one side and "51" on the other side.
Olanzapine Orodispersible Tablets 10 mg: Yellow colored, circular, flat faced beveled edge tablets, debossed with "C" on one side and "52" on the other side.

CLINICAL PARTICULARS:
Therapeutic indications
Adults
Olanzapine is indicated for the treatment of schizophrenia.
Olanzapine is effective in maintaining the clinical improvement during continuation therapy in patients who have shown an initial treatment response.
Olanzapine is indicated for the short-term treatment of acute manic episode associated with Bipolar I disorder. Olanzapine is indicated for preventing recurrence of manic, mixed or depressive episodes in Bipolar I Disorder.

Posology and method of administration
Adults
Schizophrenia: The recommended starting dose for olanzapine is 10 mg/day.
Manic episode: The starting dose is 15 mg as a single daily dose in monotherapy or 10 mg daily in combination therapy.
Preventing recurrence in bipolar disorder: The recommended starting dose is 10 mg/day. For patients who have been receiving olanzapine for treatment of manic episode, continue therapy for preventing recurrence at the same dose. If a new manic, mixed, or depressive episode occurs, olanzapine treatment should be continued (with dose optimisation as needed), with supplementary therapy to treat mood symptoms.
During treatment for schizophrenia, manic episode, and recurrence prevention in bipolar disorder, daily dosage may subsequently be adjusted on the basis of individual clinical status within the range 5-20 mg/day. An increase to a dose greater than the recommended starting dose is advised only after appropriate clinical reassessment and should generally occur at intervals of not less than 24 hours.
Olanzapine can be given without regard for meals, as absorption is not affected by food. Gradual tapering of the dose should be considered when discontinuing Olanzapine.
Olanzapine Orodispersible tablet should be placed in the mouth, where it will rapidly disperse in saliva, so it can be easily swallowed. Removal of the intact orodispersible tablet from the mouth is difficult. Since the orodispersible tablet is fragile, it should be taken immediately on opening the blister. Alternatively, it may be dispersed in a full glass of water or other suitable beverage (orange juice, apple juice, milk, or coffee) immediately before administration.
Olanzapine orodispersible tablet is bioequivalent to olanzapine tablets, with a similar rate and extent of absorption. It has the same dosage and frequency of administration as Olanzapine tablets. Olanzapine Orodispersible tablets may be used as an alternative to Olanzapine tablets.

Paediatric population
Olanzapine is not recommended for use in children and adolescents below 18 years. A greater magnitude of weight gain, lipid and prolactin alterations has been reported in adolescent patients than in adult patients
Elderly
A lower starting dose (5 mg/day) is not routinely indicated but should be considered for those 65 and over.
Renal and/or hepatic impairment
A lower starting dose (5 mg) should be considered for such patients. In cases of moderate hepatic insufficiency (cirrhosis, Child-Pugh class A or B), the starting dose should be 5 mg and only increased with caution.
Gender The starting dose and dose range need not be routinely altered for female patients relative to male patients.
Smokers
The starting dose and dose range need not be routinely altered for non-smokers relative to smokers. When more than one factor is present which might result in slower metabolism (female gender, geriatric age, non-smoking status), consideration should be given to decreasing the starting dose. Dose escalation, when indicated, should be conservative in such patients.

Contraindications
Hypersensitivity to the active substance or to any of the excipients. Patients with known risk of narrow-angle glaucoma.

Special warnings and precautions for use
During antipsychotic treatment, improvement in the patient's clinical condition may take several days to some weeks. Patients should be closely monitored during this period.

Dementia-related psychosis and/or behavioural disorders
Olanzapine is not approved for the treatment of dementia-related psychosis and/or behavioural disturbances and is not recommended for use in this particular group of patients because of an increase in mortality and the risk of cerebrovascular accident. The higher incidence of death was not associated with olanzapine dose or duration of treatment. Risk factors that may predispose this patient population to increased mortality include age > 65 years, dysphagia, sedation, malnutrition and dehydration, pulmonary conditions (e.g., pneumonia. with or without aspiration), or concomitant use of benzodiazepines. However, the incidence of death was higher in olanzapine-treated than in placebo-treated patients independent of these risk factors.
Cerebrovascular adverse events (CVAE e.g., stroke, transient ischemic attack), including fatalities, were reported. There was a 3-fold increase in CVAE in patients treated with olanzapine. All olanzapine treated patients who experienced a cerebrovascular event had pre-existing risk factors. Age > 75 years and vascular/mixed type dementia were identified as risk factors for CVAE in association with olanzapine treatment.

Parkinson's disease
The use of olanzapine in the treatment of dopamine agonist associated psychosis in patients with Parkinson's disease is not recommended. Worsening of Parkinsonian symptomatology and hallucinations were reported very commonly and more frequently, and olanzapine was not more effective than placebo in the treatment of psychotic symptoms. In these trials, patients were initially required to be stable on the lowest effective dose of anti-Parkinsonian medicinal products (dopamine agonist) and to remain on the same anti-Parkinsonian medicinal products and dosages throughout the study. Olanzapine was started at 2.5 mg/day and titrated to a maximum of 15 mg/day based on investigator judgement.

Neuroleptic Malignant Syndrome (NMS)
NMS is a potentially life-threatening condition associated with antipsychotic medicinal product. Rare cases reported as NMS have also been received in association with olanzapine. Clinical manifestations of NMS are hyperpyrexia, muscle rigidity, altered mental status, and evidence of autonomic instability (irregular pulse or blood pressure, tachycardia, diaphoresis, and cardiac dysrhythmia). Additional signs may include elevated creatine phosphokinase, myoglobinuria (rhabdomyolysis), and acute renal failure. If a patient develops signs and symptoms indicative of NMS, or presents with unexplained high fever without additional clinical manifestations of NMS, all antipsychotic medicines, including olanzapine must be discontinued.

Lipid alterations
Undesirable alterations in lipids have been observed in olanzapine treated patients. Lipid alterations should be managed as clinically appropriate, particularly in dyslipidemic patients and in patients with risk factors for the development of lipids disorders. Patients treated with any antipsychotic agents, including Olanzapine Tablets, should be monitored regularly for lipids, in accordance with utilised antipsychotic guidelines.

Anticholinergic activity
While olanzapine demonstrated anticholinergic activity in vitro, experience during the clinical trials revealed a low incidence of related events. However, as clinical experience with olanzapine in patients with concomitant illness is limited, caution is advised when prescribing for patients with prostatic hypertrophy, or paralytic ileus and related conditions.

Hepatic function
Transient, asymptomatic elevations of hepatic transaminases, ALT, AST have been seen commonly, especially in early treatment. Caution should be exercised in patients with elevated ALT and/or AST, in patients with signs and symptoms of hepatic impairment, in patients with pre-existing conditions associated with limited hepatic functional reserve, and in patients who are being treated with potentially hepatotoxic medicines. In the event of elevated ALT and/or AST during treatment, follow-up should be organised and dose reduction should be considered. In cases where hepatitis (including hepatocellular, cholestatic or mixed liver injury) has been diagnosed, olanzapina treatment should be discontinued.

Neutropenia
Caution should be exercised in patients with low leukocyte and/or neutrophil counts for any reason, in patients receiving medicines known to cause neutropenia, in patients with a history of drug-induced bone marrow depression/toxicity, in patients with bone marrow depression caused by concomitant illness,

radiation therapy or chemotherapy and in patients with hypereosinophilic conditions or with myeloproliferative disease. Neutropenia has been reported commonly when olanzapine and valproate are used concomitantly

Discontinuation of treatment
Acute symptoms such as sweating, insomnia, tremor, anxiety, nausea, or vomiting have been reported very rarely when olanzapine is stopped abruptly.

QT interval
Clinically meaningful QTc prolongations (Fridericia QT correction (QTcF) ≥ 500 milliseconds (msec) at any time post baseline in patients with baseline QTcF <500 msec) were uncommon in patients treated with olanzapine, with no significant differences in associated cardiac events compared to placebo. However, as with other antipsychotics, caution should be exercised when olanzapine is prescribed with medicines known to increase QTc interval, especially in the elderly, in patients with congenital long QT syndrome, congestive heart failure, heart hypertrophy, hypokalaemia or hypomagnesaemia.

Thromboembolism
Temporal association of olanzapine treatment and venous thromboembolism has very rarely been reported. A causal relationship between the occurrence of venous thromboembolism and treatment with olanzapine has not been established. However, since patients with schizophrenia often present with acquired risk factors for venous thromboembolism all possible risk factors of VTE e.g. immobilisation of patients, should be identified and preventive measures undertaken

General CNS activity
Given the primary CNS effects of olanzapine, caution should be used when it is taken in combination with other centrally acting medicines and alcohol. As it exhibits in vitro dopamine antagonism, olanzapine may antagonize the effects of direct and indirect dopamine agonists.

Seizures
Olanzapine should be used cautiously in patients who have a history of seizures or are subject to factors which may lower the seizure threshold. Seizures have been reported to occur rarely in patients when treated with olanzapine. In most of these cases, a history of seizures or risk factors for seizures were reported.

Tardive Dyskinesia
Olanzapine was associated with a statistically significant lower incidence of treatment emergent dyskinesia. However the risk of tardive dyskinesia increases with long term exposure, and therefore if signs or symptoms of tardive dyskinesia appear in a patient on Olanzapine, a dose reduction or discontinuation should be considered. These symptoms can temporally deteriorate or even arise after discontinuation of treatment.

Postural hypotension
Postural hypotension was infrequently observed in the elderly in olanzapine clinical trials. As with other antipsychotics, it is recommended that blood pressure is measured periodically in patients over 65 years.

Use In children and adolescents under 18 years of age
Olanzapine is not indicated for use in the treatment of children and adolescents. Studies in patients aged 13-17 years showed various adverse reactions, including weight gain, changes in metabolic parameters and increases in prolactin levels. Long-term outcomes associated with these events have not been studied and remain unknown.

Phenylalanine
Olanzapine Orodispersible Tablet contains aspartame, which is a source of phenylalanine and is unsuitable for phenylketonurics.

Mannitol
Olanzapine Orodispersible Tablet contains mannitol.

Hyperglycemia and Diabetes Mellitus
Hyperglycemia in some cases extreme and associated with ketoacidosis or hyperosmolar coma or death, has been reported in patients treated with atypical antipsychotics. Assessment of the relationship between atypical antipsychotics use and glucose abnormalities is complicated by the possibility of an increased background risk of diabetes mellitus in patients with schizophrenia and the increasing incidence of diabetes mellitus in the general population. Given this confounders, the relationship between atypical antipsychotic use and hyperglycemia-related adverse events is not completely understood. However, epidemiological studies suggest an increased risk of treatment-emergent hyperglycemia-related adverse events in patients treated with the atypical antipsychotics. Precise risk estimates for hyperglycemia-related adverse events in patients treated with atypical antipsychotics are not available.
Patients with an established diagnosis of diabetes mellitus who are started on atypical antipsychotics should be monitored regularly for worsening of glucose control. Patients with risk factors for diabetes mellitus (e.g. obesity, family history of diabetes) who are starting treatment with atypical antipsychotics should undergo fasting blood glucose testing at the beginning of treatment and periodically during treatment. Any patient treated with atypical antipsychotics should be monitored for symptoms of hyperglycemia including polydipsia, polyuria, polyphagia, and weakness. Patients who develop symptoms of hyperglycemia during treatment with atypical antipsychotics should undergo fasting blood glucose testing. In some cases, hyperglycemia has resolved when the atypical antipsychotic was discontinued; however, some patients required continuation of anti-diabetic treatment despite discontinuation of the suspect drug.

Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS):
Drug reaction with Eosinophilia and Systemic Symptoms (DRESS) has been reported with olanzapine exposure. DRESS consists of a combination of three or more of the following: cutaneous reaction (such as rash or exfoliative dermatitis), eosinophilia, fever, lymphadenopathy and one or more systemic complications such as hepatitis, nephritis, pneumonitis, myocarditis, and pericarditis. Discontinue olanzapine if DRESS is suspected.

Ability to drive and use machine
Because Olanzapine may cause somnolence patients should be cautioned about operating hazardous machinery including motor vehicles.

Interaction with other Medicaments
Caution should be exercised in patients who receive medicinal products that can cause central nervous system depression.

Potential interactions affecting olanzapine:
Since olanzapine is metabolised by CYP1A2, substances that can specifically induce or inhibit this isoenzyme may affect the pharmacokinetics of olanzapine.

Induction of CYP1A2: The metabolism of olanzapine may be induced by smoking and carbamazepine, which may lead to reduced olanzapine concentrations. Only slight to moderate increase in olanzapine clearance has been observed. The clinical consequences are likely to be limited, but clinical monitoring is recommended and an increase of olanzapine dose may be considered if necessary.

Inhibition of CYP1A2: Fluvoxamlne, a specific CYP1A2 inhibitor, has been shown to significantly inhibit the metabolism of olanzapine. A lower starting dose of olanzapine should be considered in patients who are using fluvoxamine or any other CYP1A2 inhibitors, such as ciprofloxacin or ketconazole. A decrease in the dose of olanzapine should be considered if treatment with an inhibitor of CYP1A2 is initiated.

Decreased bioavailability: Activated charcoal reduces the bioavailability of oral olanzapine and should be taken atleast 2 hours before or after olanzapine. Fluoxetine (a CYP2D6 inhibitor), single doses of antacid (aluminium, magnesium) or cimetidine have not been found to significantly affect the pharmacokinetics of olanzapine.

Potential for olanzapine to affect other medicinal products: Olanzapine may antagonize the effects of direct and indirect dopamine agonists.
Olanzapine does not inhibit the main CYP450 Isoenzymes in vitro (e.g. 1A2, 2D6, 2C9, 2C19, 3A4). Thus no particular interaction is expected as verified through in vivo studies where no inhibition of metabolism of the following active substances was found: tricyclic antidepressant (representing mostly CYP2D6 pathway), warfarin (CYP2C9), theophylline (CYP1A2) or diazepam (CYP3A4 and 2C19).
Olanzapine showed no interaction when co-administered with lithium or biperiden. Therapeutic monitoring of valproate plasma levels did not indicate that valproate dosage adjustment is required after the introduction of concomitant olanzapine.

Pregnancy and lactation.
Pregnancy

Patients should be advised to notify their physician if they become pregnant or intend to become pregnant during treatment with olanzapine. Olanzapine should be used in pregnancy only if the potential benefit justifies the potential risk to the foetus.
Spontaneous reports have been very rarely received on tremor, hypertonia, lethargy and sleepiness, in infants born to mothers who had used olanzapine during the 3rd trimester.

Breast feeding
In breast feeding, healthy women, olanzapine was excreted in breast milk. Mean infant exposure (mg/kg) at steady-state was estimated to be 1.8% of the maternal olanzapine dose (mg/kg). Patients should be advised not to breast feed an infant if they are taking olanzapine.

Neonates exposed to antipsychotic drugs during the third trimester of pregnancy are at risk for extrapyramidal and/or withdrawal symptoms following delivery. There have been reports of agitation, hypertonia, hypotonic, tremor, somnolence, respiratory distress, and feeding disorder in these neonates. These complications have varied in severity; while in some cases symptoms have been self-limited, in other cases neonates have required intensive care unit support and prolonged hospitalisation.
Olanzapine should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus.

Undesirable effects
Adults
The most frequently observed adverse reactions associated with the use of olanzapine were somnolence, weight gain, eosinophilia, elevated prolactin, cholesterol, glucose and triglyceride levels, glucosuria, increased appetite, dizziness, akathisia, parkinsonism, dyskinesia, orthostatic hypotension, anticholinergic effects, transient asymptomatic elevations of hepatic transaminases, rash, asthenia, fatigue and oedema.
The following table lists the adverse reactions. Within each frequency grouping, adverse reactions are presented in order of decreasing seriousness. The frequency terms listed are defined as follows: Very

A/s: 210 x 420 mm ■ Black Booklet Size: 35 x 60 mm

 AUROBINDO Packaging Development		Product Name	Component	Item Code	Date & Time
		OLANGEM OD	Leaflet	P1511215	21.05.2019 & 9:10 am
		Customer / Country	Version No.	Reason Of Issue	Reviewed / Approved by
Team Leader	Kiran	Malaysia_SEZ	13	Commercial	
Initiator	Krishna	Dimensions	Colours : 01		
Artist:		210 x 420 mm	 11215		
		Pharmacode			
Additional Information :		11215			Sign / Date

common, common uncommon, rare very rare, not known (cannot be estimated from the data available).

Very Common	Common	Uncommon	Very rare	Not known
Blood and the lymphatic system disorders				
	Eosinophilia	Leucopenia Neutropenia		Thrombocytopenia
Immune system disorders				
				Allergic reaction
Metabolism and nutrition disorders				
Weight gain	Elevated cholesterol levels Elevated glucose levels Elevated triglyceride Levels Glucosuria Increased appetite			Development or exacerbation of diabetes occasionally associated with ketoacidosis or coma, including some fatal cases Hypothermia
Nervous system disorders				
Somnolence	Dizziness Akathisia Parkinsonism Dyskinesia			Seizures where in most cases a history of seizures or risk factors for seizures were reported Neuroleptic malignant syndrome Dystonia (including oculogyration) Tardive dyskinesia Discontinuation symptoms Restless legs syndrome
Cardiac disorders				
	Bradycardia QTc prolongation			Ventricular tachycardia/fibrillation, sudden death
Vascular disorders				
	Orthostatic hypotension			Thromboembolism (including pulmonary embolism and deep vein thrombosis)
Gastrointestinal disorders				
	Mild, transient Anticholinergic effects including constipation and dry mouth			Pancreatitis
Hepato-biliary disorders				
	Transient, asymptomatic elevations of hepatic transaminases (ALT, AST), especially in early treatment			Hepatitis (including hepatocellular, cholestatic or mixed liver injury)
Skin and subcutaneous tissue disorders				
	Rash	Photosensitivity Reaction Alopecia.	Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS)	
Musculoskeletal and connective tissue disorders				
				Rhabdomyolysis
Renal and urinary disorders				
				Urinary hesitation Urinary retention
Reproductive system and breast disorders				
				Priapism
General disorders and administration site conditions				
	Asthenia Fatigue Oedema			
Investigations				
Elevated plasma prolactin levels		High creatine phosphokinase Increased total bilirubin		Increased alkaline phosphatase
Respiratory Thoracic and Mediastinal Disorders				
				Sleep apnoea

* Atypical antipsychotic drugs, such as Olanzapine, have been associated with cases of sleep apnoea, with or without concomitant weight gain. In patients who have a history of or are at risk for sleep apnoea, Olanzapine Orodispersible Tablets should be prescribed with caution.

Additional information on special populations

In elderly patients with dementia, olanzapine treatment was associated with a higher incidence of death and cerebrovascular adverse reactions Very common adverse reactions associated with the use of olanzapine in this patient group were abnormal gait and falls. Pneumonia, increased body temperature, lethargy, erythema, visual hallucinations and urinary incontinence were observed commonly. Olanzapine administered with lithium or valproate resulted in increased levels of tremor, dry mouth, increased appetite, and weight gain. Speech disorder was also reported commonly. During treatment with olanzapine in combination with lithium or divalproex, an increase of body weight occurs. Children and adolescents Olanzapine is not indicated for the treatment of children and adolescent patients below 18 years. The following are adverse reactions reported with a greater frequency in adolescent patients (aged 13-17 years) than in adult patients or adverse reactions only identified in adolescent patients. Clinically significant weight gain (≥7%) appears to occur more frequently in the adolescent population compared to adults with comparable exposures. The magnitude of weight gain and the proportion of adolescent patients who had clinically significant weight gain were greater with long-term exposure (at least 24 weeks) than with short-term exposure. Within each frequency grouping, adverse reactions are presented in order of decreasing seriousness. The frequency terms listed are defined as follows: Very common, common.

Metabolism and nutrition disorders <i>Very common:</i> Weight gain, elevated triglyceride levels, increased appetite. <i>Common:</i> Elevated cholesterol levels.
Nervous system disorders <i>Very common:</i> Sedation (including: hypersomnia, lethargy, somnolence).
Gastrointestinal disorders <i>Common:</i> Dry mouth.
Hepato-biliary disorders <i>Very common:</i> Elevations of hepatic transaminases ALT/AST
Investigations <i>Very common:</i> Decreased total bilirubin increased GGT, elevated plasma prolactin levels.

Overdosage

Very common symptoms in overdose (>10% incidence) include tachycardia, agitation/aggressiveness, dysarthria, various extrapyramidal symptoms, and reduced level of consciousness ranging from sedation to coma.

Other medically significant sequelae of overdose include delirium, convulsion, coma, possible neuroleptic malignant syndrome, respiratory depression, aspiration, hypertension or hypotension, cardiac arrhythmias (<2% of overdose cases) and cardiopulmonary arrest. Fatal outcomes have been reported for acute overdoses as low as 450 mg, but survival has also been reported following acute overdose of approximately 2 g of oral olanzapine.

Management of Overdose

There is no specific antidote for olanzapine. Induction of emesis is not recommended. Standard procedures for management of overdose may be indicated (i.e., gastric lavage, administration of activated charcoal). The concomitant administration of activated charcoal reduces the oral bioavailability of olanzapine by 50 to 60%.

Symptomatic treatment and monitoring of vital organ function should be instituted, including treatment of hypotension and circulatory collapse and support of respiratory function. The use of epinephrine, dopamine, or other sympathomimetic agents with beta-agonist activity is restricted, since beta stimulation may worsen hypotension. Cardiovascular monitoring is necessary to detect possible arrhythmias. Close medical supervision and monitoring should continue until the patient recovers.

PHARMACOLOGICAL PROPERTIES

Pharmacodynamic properties

Olanzapine is an antipsychotic, antimanic and mood stabilising agent that demonstrates a broad pharmacologic profile across a number of receptor systems. Olanzapine exhibits a range of receptor affinities for serotonin 5HT2A/2C, 5HT3, 5HT6; dopamine D1, D2, D3, D4, D5; cholinergic muscarinic receptors m1-m5; α1 adrenergic; and histamine H1 receptors. Olanzapine reduced a conditioned avoidance response, a test indicative of antipsychotic activity, at doses below those producing catalepsy, an effect indicative of motor side-effects. Unlike some other antipsychotic agents, olanzapine increases responding in an 'anxiolytic' test.

Paediatric population

During treatment with olanzapine, adolescents gained significantly more weight compared with adults. The magnitude of changes in fasting total cholesterol, LDL cholesterol, triglycerides and prolactin were greater in adolescents than in adults.

Pharmacokinetic properties

Olanzapine is well absorbed after oral administration, reaching peak plasma concentrations within 5 to 8 hours. The absorption is not affected by food. Absolute oral bioavailability relative to intravenous administration has not been determined.

Olanzapine is metabolised in the liver by conjugative and oxidative pathways. The major circulating metabolite is the 10-N-glucuronide, which does not pass the blood brain barrier. Cytochromes P450-CYP1A2 and P450-CYP2D6 contribute to the formation of the N-desmethyl and 2-hydroxymethyl metabolites. The predominant pharmacologic activity is from the parent, olanzapine. After oral administration, the mean terminal elimination half-life of olanzapine in healthy subjects varied on the basis of age and gender.

The pharmacokinetic variability observed in the elderly is within the range for the non-elderly. The plasma clearance of olanzapine is lower in elderly versus young subjects, in females versus males, and in nonsmokers versus smokers. However, the magnitude of the impact of age, gender, or smoking on olanzapine clearance and half-life is small in comparison to the overall variability between individuals. The plasma protein binding of olanzapine was about 93% over the concentration range of about 7 to about 1,000 ng/ml. Olanzapine is bound predominantly to albumin and α1-acid-glycoprotein. The pharmacokinetics of olanzapine is similar between adolescents and adults. The average olanzapine exposure was approximately 27% higher in adolescents. Demographic differences between the adolescents and adults include a lower average body weight and fewer adolescents were smokers. Such factors possibly contribute to the higher average exposure observed in adolescents.

PHARMACEUTICAL PARTICULARS

List of excipients

Mannitol, Polacrillin potassium, Crospovidone (Type A), Silica Colloidal Anhydrous, Aspartame, Cellulose microcrystalline (PH-112), Sodium Stearyl Fumarate, Art Pineapple.

Incompatibilities

None known.

Shelf life

Please refer outer package for expiry date.

Special precautions for storage

Store below 30°C. Store in a original pack in order to protect from light and moisture.

Nature and contents of container

OLANGEM OD 5 & OLANGEM OD 10: The tablets are packed in Cold form with dessicant-Aluminium blister pack with 7 tablets per blister.

Each carton contains 4 x7's tablets.

Manufactured by



AUROBINDO

Aurobindo Pharma Ltd.,
Unit VII, SEZ, TSIC, Plot. No.S1, Survey No's:411/P, 425/P, 434/P, 435/P & 458/P
Green Industrial Park, Polepally Village,
Jedcherla Mandal, Mahaboobnagar District,
Telangana State.

Product registration holder:

Healol Pharmaceuticals Sdn. Bhd.,
No. 74-3, Jalan Wangsa Delima 6,
KLSC Wangsa Maju, 53300 Kuala Lumpur, Malaysia.

DATE OF PREPARATION OF THIS LEAFLET

May 2019.